

Vaginal Suspension Surgery

Patient Information · Lifting the top of the vagina using your own tissue, with no mesh

WARWIKI

Vaginal suspension surgery repairs prolapse of the **top of the vagina or the uterus**. It attaches the top of the vagina to strong ligaments in your pelvis. It uses your **own tissue**, with no mesh. It is done through the vagina, with no cut in the belly.

About This Procedure

The top of the vagina is stitched to a firm ligament in the pelvis to hold it up. There are two common types:

- **Uterosacral ligament suspension (USLS):** the top is attached to the ligaments that support the uterus.
- **Sacrospinous ligament fixation (SSLF):** the top is attached to a ligament deep in the pelvis, toward one side.

Both use **your own tissue** and stitches, not mesh. They are often done with a hysterectomy or with repairs of the front or back wall.

Benefits and Risks in Numbers

5 in 100

need more prolapse treatment within 2 years, with either type

7–12 in 100

need extra care for buttock or leg pain (more often after SSLF)

3 in 100

have a blocked ureter during USLS, usually fixed during surgery

Numbers are averages from published studies. Your own risk may differ; ask your team.

How It Compares

Sacrocolpopexy lifts the vagina with mesh placed through the belly. It may last longer, but it adds a belly operation and mesh risks. Your surgeon will help you weigh the choices.

Learn the Terms

Apex or vault

The top of the vagina. It can drop with prolapse.

Suspension

Attaching the top of the vagina to a strong ligament.

USLS

Uterosacral ligament suspension.

SSLF

Sacrospinous ligament fixation.

Native tissue

Your own tissue. This repair uses no mesh.

Ureter

The tube that carries urine from the kidney to the bladder.

Cystoscopy

A look inside the bladder with a small camera.

Will it hurt? You will be asleep or numb during surgery. Afterward, expect soreness in the vagina. Some people have **buttock or upper leg pain** for a few days to weeks, most often after SSLF. It usually fades on its own.

First day

Walk soon after surgery. Your bladder emptying is checked.

First weeks

Spotting and discharge are normal. Return to activity, lifting and exercise at your own pace.

Until cleared

Nothing in the vagina and no sex until your team says so, often about 6 weeks.

Follow-up visit

Your team checks healing, pain and how the repair holds.

How to Get Ready

- Follow the instructions about eating, drinking and blood thinners.
- Ask which other repairs, or a leakage procedure, will be done.
- Stop smoking. Arrange a ride and some help at home.

What Happens During Surgery

- 1 You get general or spinal anesthesia, and antibiotics.
- 2 Through the vagina, the top of the vagina is stitched to a strong ligament. Other planned repairs are done.
- 3 A **cystoscopy** checks that urine flows from both kidneys.
- 4 A catheter, and sometimes gauze in the vagina, may stay in for a short time.

Call the office that did your surgery if you have:

- A temperature of **100.4 °F (38 °C)** or higher, or chills
- **Severe or shooting buttock or leg pain**, or new numbness
- New pain in your side or back, or urine leaking from the vagina

Go to an emergency department if:

- You **cannot urinate** for several hours and your bladder feels full or painful
- Bleeding that soaks a pad in an hour
- You cannot reach the office and are worried

Three Things to Remember

1. A vaginal suspension lifts the top of the vagina with your own tissue, through the vagina.
2. Mild buttock ache is common and usually fades. Severe or shooting pain needs a call.
3. Walk from the first day. Wait for your team's OK before sex.